

LEGISLATURE OF THE STATE OF IDAHO
Sixty-eighth Legislature Second Regular Session - 2026

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 753

BY HEALTH AND WELFARE COMMITTEE

AN ACT

RELATING TO ASSERTIVE COMMUNITY TREATMENT; PROVIDING LEGISLATIVE INTENT; AMENDING CHAPTER 22, TITLE 56, IDAHO CODE, BY THE ADDITION OF A NEW SECTION 56-2208, IDAHO CODE, TO PROVIDE LEGISLATIVE APPROVAL FOR STATE PLAN AMENDMENTS REGARDING ASSERTIVE COMMUNITY TREATMENT; AMENDING TITLE 56, IDAHO CODE, BY THE ADDITION OF A NEW CHAPTER 25, TITLE 56, IDAHO CODE, TO DEFINE TERMS, TO ESTABLISH PROVISIONS REGARDING OBJECTIVES AND ELIGIBILITY FOR ASSERTIVE COMMUNITY TREATMENT, TO PROVIDE FOR THE ESTABLISHMENT OF ASSERTIVE COMMUNITY TREATMENT PROGRAMS AND TEAMS, TO ESTABLISH PROVISIONS REGARDING MEASURES AND OUTCOMES, TO ESTABLISH PROVISIONS REGARDING FIDELITY STANDARDS AND PAYMENT FOR SERVICES, AND TO PROVIDE RULEMAKING AUTHORITY; AND DECLARING AN EMERGENCY.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. LEGISLATIVE INTENT. It is the intent of the Legislature that assertive community treatment services be generally targeted to individuals with serious and persistent mental illness who experience significant functional impairment and who have not been adequately served by less intensive clinic-based or episodic services.

SECTION 2. That Chapter 22, Title 56, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW SECTION, to be known and designated as Section 56-2208, Idaho Code, and to read as follows:

56-2208. LEGISLATIVE APPROVAL -- ASSERTIVE COMMUNITY TREATMENT. The department of health and welfare is authorized to seek and shall submit a state plan amendment to the centers for medicare and medicaid services no later than March 1, 2026, to provide assertive community treatment as a rehabilitative service in accordance with the provisions of chapter 25, title 56, Idaho Code.

SECTION 3. That Title 56, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW CHAPTER, to be known and designated as Chapter 25, Title 56, Idaho Code, and to read as follows:

CHAPTER 25
ASSERTIVE COMMUNITY TREATMENT

56-2501. DEFINITIONS. As used in this chapter:
(1) "Assertive community treatment" or "ACT" means a multidisciplinary, community-based behavioral health service model that provides intensive, comprehensive, and individualized treatment and support services to eligible participants with serious and persistent mental illness. An ACT team shall:

- 1 (a) Be available to provide services twenty-four (24) hours per day,
- 2 seven (7) days per week;
- 3 (b) Deliver services primarily in community-based settings as clini-
- 4 cally appropriate;
- 5 (c) Provide a full range of treatment, rehabilitation, and support ser-
- 6 vices through a team based approach;
- 7 (d) Address complex behavioral health needs through integrated psychi-
- 8 atric, medical, substance use, rehabilitative, and peer support ser-
- 9 vices;
- 10 (e) Operate in accordance with applicable state requirements and the
- 11 adopted fidelity model; and
- 12 (f) Include an interdisciplinary team composed of, at a minimum, or as
- 13 otherwise determined by the adopted fidelity model:
- 14 (i) Licensed nursing staff;
- 15 (ii) Licensed practitioners practicing within the scope of their
- 16 licensure;
- 17 (iii) Master's or doctoral level mental health professionals;
- 18 (iv) Clinical supervision provided by a master's or doctoral
- 19 level mental health professional within the scope of their licen-
- 20 sure;
- 21 (v) Paraprofessional staff qualified to provide skills-build-
- 22 ing, psychosocial rehabilitation, and case management services;
- 23 (vi) Certified peer support specialists;
- 24 (vii) Staff trained and qualified to provide assessment and
- 25 treatment for substance use disorders; and
- 26 (viii) Administrative support staff.
- 27 (2) "Department" means the Idaho department of health and welfare.

28 56-2502. ASSERTIVE COMMUNITY TREATMENT -- OBJECTIVES -- ELIGIBIL-
 29 ITY. (1) Assertive community treatment is intensive nonresidential treat-
 30 ment and rehabilitative mental health services provided to eligible par-
 31 ticipants with serious and persistent mental illness. Assertive community
 32 treatment is delivered through a single, fixed entity per region responsible
 33 for treatment, rehabilitation, and support needs for participants. Ser-
 34 vices are offered twenty-four (24) hours per day, seven (7) days per week,
 35 in community-based settings or other settings as clinically appropriate and
 36 consistent with the state's chosen assertive community treatment fidelity
 37 model. The objectives of assertive community treatment include:

- 38 (a) Preventing or reducing symptoms or behaviors that may result in the
- 39 need for recurrent use of inpatient services or high-cost intervention
- 40 systems, including law enforcement or emergency medical systems inter-
- 41 actions, or incarceration; and
- 42 (b) Increasing the skills and behaviors that enhance the participant's
- 43 ability to remain in the community.

44 (2) To ensure efficient use of public resources and appropriate target-
 45 ing of services, a participant is eligible for assertive community treatment
 46 services only if the participant meets the eligibility criteria listed in
 47 the state's chosen assertive community treatment fidelity model and:

1 (a) Has a pattern of frequent use of crisis services, repeated hospi-
2 talizations, incarceration, or other high-cost services related to the
3 diagnosed disorder;

4 (b) Has a primary diagnosis of serious and persistent mental illness,
5 as defined in section 39-3122, Idaho Code, consistent with the most
6 recent diagnostic and statistical manual of mental disorders, and
7 demonstrates significant functional impairment that substantially in-
8 terferes with the individual's ability to function independently in one
9 (1) or more major life activities; and

10 (c) Has symptoms and behaviors as evidenced by one (1) or more of the
11 following:

12 (i) A history of alcohol and drug abuse in combination with psy-
13 chiatric symptoms or other serious medical or physical problems;

14 (ii) A pattern of isolation with extremely poor or nonexistent so-
15 cial or family support;

16 (iii) A pattern of inability to provide for basic needs for food,
17 clothing, and shelter;

18 (iv) A pattern of urgent and severe psychiatric and other concomi-
19 tant medical difficulties; or

20 (v) Has failed to remain engaged in or respond to conventional
21 services, such as case management, medication, outpatient treat-
22 ment, or day programs.

23 (3) Additional requirements in selecting participants for assertive
24 community treatment shall include at least one (1) of the following:

25 (a) A written opinion by a licensed mental health professional that the
26 participant needs mental health services that cannot be met with other
27 available community-based services or is likely to experience a mental
28 health crisis or require a more restrictive setting if assertive commu-
29 nity treatment is not provided; or

30 (b) Whether or not other community-based services that are appropriate
31 to the participant's needs and reasonably available would be equally or
32 more effective in treating the participant, as evidenced by consistent
33 and extensive efforts to treat the participant.

34 (4) Assertive community treatment services shall be provided as a re-
35 habilitative service under the state medicaid plan, consistent with federal
36 medicaid law and implementing regulations. The purpose of the service is to
37 restore, maintain, and improve functional capacity, reduce reliance on in-
38 stitutional care, and support individuals in living successfully in inte-
39 grated community settings.

40 (5) (a) Assertive community treatment services shall be available to
41 eligible individuals who meet medical necessity and program eligibil-
42 ity criteria established consistent with the state medicaid plan and
43 applicable federal requirements.

44 (b) The department of health and welfare or its designee shall adminis-
45 ter assertive community treatment services in a manner consistent with
46 the adopted fidelity model, including required staffing ratios and team
47 composition, to ensure program integrity and effectiveness.

48 (c) The number of participants served by assertive community treatment
49 teams shall be determined based on demonstrated clinical need, provider

1 capacity, and compliance with fidelity standards and shall not be sub-
2 ject to statutory enrollment caps.

3 56-2503. ASSERTIVE COMMUNITY TREATMENT PROGRAM -- TEAM. (1) The de-
4 partment of health and welfare or its designee shall establish an assertive
5 community treatment program consistent with a standardized, nationally rec-
6 ognized assertive community treatment framework and the state's adopted fi-
7 delity model and shall adopt rules, subject to legislative approval, regard-
8 ing staffing and service provision that address:

9 (a) Team composition and staffing patterns based on the anticipated
10 participant population and in accordance with the requirements of this
11 section;

12 (b) Training and supervision, including initial and ongoing cross-dis-
13 ability training, if applicable;

14 (c) Communication among team members regarding participant condition,
15 supported by the assignment of daily staff responsibilities and regu-
16 lar, frequent staffing;

17 (d) Days and hours of operation;

18 (e) An after-hours plan, including on-call coverage and linkages with
19 appropriate emergency services;

20 (f) Participant selection procedures and criteria consistent with the
21 provisions of section 56-2502, Idaho Code;

22 (g) Description of service provision by an assertive community treat-
23 ment team that aligns with the adopted assertive community treatment
24 fidelity model; and

25 (h) Policies regarding quality assurance and quality improvement, in-
26 cluding outcome measures.

27 (2) The assertive community treatment team shall provide service
28 twenty-four (24) hours per day, seven (7) days per week, and be prepared to
29 carry out a full range of treatment functions. The team shall be interdis-
30 ciplinary and structured to meet the complex needs of participants wherever
31 and whenever clinically appropriate and shall include the staffing roles re-
32 quired pursuant to section 56-2501, Idaho Code, unless otherwise determined
33 by the fidelity model.

34 (3) Each member of the assertive community treatment team shall meet
35 applicable Idaho licensure and qualification requirements and shall be su-
36 pervised consistent with their role and scope of practice.

37 (4) The participant-to-staff ratio shall be based on the needs of the
38 participants for whom the team is assigned responsibility. The participant-
39 to-staff ratio shall be ten (10) participants to one (1) staff person. In no
40 event shall the ratio exceed twelve (12) participants to one (1) staff per-
41 son. Staffing levels and team roles shall be consistent with the adopted as-
42 sertive community treatment fidelity model chosen by the state.

43 (5) The team shall provide at least two (2) contacts per week to partic-
44 ipants experiencing severe symptoms or significant problems in daily liv-
45 ing. When a participant is unable to be located or declines contact, the team
46 shall make and document reasonable outreach efforts consistent with the par-
47 ticipant's individualized treatment plan. Each participant shall be seen
48 face-to-face by an employee or contractor in accordance with the schedule
49 specified in the participant's individualized treatment plan.

(6) Assertive community treatment services shall be delivered as a distinct, integrated service model by a single interdisciplinary team that assumes primary responsibility for the assessment, planning, coordination, and delivery of all assertive community treatment services, consistent with the adopted fidelity model. Assertive community treatment services shall be delivered through the integrated team based model and shall not be provided across multiple provider entities or delivery systems in a manner that undermines model fidelity. When a participant requires specialized services that are not available through the assigned assertive community treatment team, such services may be obtained through other licensed or medicaid-enrolled providers; provided that the delivery of individual service components outside the integrated assertive community treatment model does not constitute assertive community treatment.

56-2504. MEASURES AND OUTCOMES. (1) Assertive community treatment objectives for a participant shall be addressed by activities designed to:

- (a) Promote symptom stability and the appropriate use of medication;
- (b) Restore personal, community living, and social skills necessary for independent functioning;
- (c) Promote and maintain physical health;
- (d) Establish access to public benefits, housing, work, and social opportunities; and
- (e) Promote and maintain the highest practicable level of functioning in the community.

(2) Assertive community treatment services shall be delivered in a recovery-oriented manner and shall focus on rehabilitative outcomes, including:

- (a) Improved functional capacity;
- (b) Increased housing stability;
- (c) Reduced hospitalizations and justice system involvement;
- (d) Enhanced community integration; and
- (e) Improved quality of life.

(3) Assertive community treatment services shall be delivered primarily in community settings where participants live, work, or receive support and shall not be dependent on clinic-based or office-based service delivery. Services shall be provided through mobile, in-person outreach and engagement that meets participants where they are, in accordance with the standards set forth in this section, to promote engagement, participation in services, and participation in community life.

(4) If a participant's needs can be adequately and appropriately addressed with an average of fewer than seven (7) face-to-face contacts per month during any three (3) consecutive month period, the participant's individualized treatment plan shall be reviewed to determine whether less intensive service alternatives may be appropriate in lieu of assertive community treatment services. If it is determined that less intensive services can meet the participant's needs over the long term, the clinical basis for that determination shall be documented, a plan for continuity of care shall be established, and the participant shall no longer be eligible for assertive community treatment services.

(5) Providers shall report standardized outcome measures as specified by the adopted assertive community treatment fidelity model, as determined by the department or its designee, which may include reductions in psychiatric hospitalizations, reductions in days of incarceration, improvements in participant housing stability, and increased engagement with family and community supports.

(6) A participant shall not be discharged from assertive community treatment services solely for failure to comply with a treatment plan or other expectations, except as otherwise provided in this section. A participant may be discharged only upon meeting at least one (1) of the following criteria:

(a) A change in the participant's residence to a location out of the service area;

(b) Incarceration or long-term hospitalization of the participant for a period exceeding thirty (30) days. In such cases, the provider shall prioritize the participant for re-enrollment in assertive community treatment services upon the participant's anticipated return to the community if the participant requests services and the level of care remains appropriate;

(c) The participant elects to withdraw from services, and documented attempts by the program to re-engage the participant have not been successful;

(d) The participant and the team determine that assertive community treatment services are no longer necessary based on the attainment of the individual treatment plan goals and that a less intensive level of care would adequately address the participant's current needs; or

(e) The participant has engaged in documented threats, acts of violence, or abusive conduct toward members of the assertive community treatment team or its contractors that jeopardize staff safety.

56-2505. FIDELITY STANDARDS AND PAYMENT FOR SERVICES. (1) To ensure accountability for state funding and measurable program performance, assertive community treatment provider teams shall achieve an initial minimum score of 3.0 on the tool for measurement of assertive community treatment or another nationally recognized assertive community treatment fidelity instrument approved by the department or its designee. Provider teams shall achieve a minimum score of 4.0 within three (3) years of initial certification. The department or its designee may grant time-limited variances from fidelity thresholds for newly established teams or teams operating in regions with documented workforce shortages, provided that a corrective action plan is implemented and progress toward full fidelity is demonstrated. Any variance granted under this subsection shall not exceed six (6) months, unless extended by the department or its designee upon a showing of substantial progress toward the corrective action plan. Variances shall not be renewed without demonstrated compliance with fidelity requirements.

(2) Assertive community treatment services reimbursed under the state medicaid plan shall be reimbursed using a single monthly assertive community treatment service unit per enrolled participant, inclusive of all rehabilitative services, outreach, coordination, travel time, and crisis response furnished during the applicable billing month, with all covered

1 services aggregated into a single unit of reimbursement. The aggregated
2 amount may be billed on service or non-service days for up to thirty (30) days
3 following the last successful contact, provided that continued outreach
4 efforts are documented and consistent with the state's adopted assertive
5 community treatment fidelity model. Services and interventions reimbursed
6 pursuant to this subsection shall be consistent with the participant's in-
7 dividualized treatment plan and may include direct services, collateral
8 contacts, crisis response, or assertive outreach. Rates shall be reviewed
9 and adjusted annually using the centers for medicare and medicaid services
10 medicare economic index to maintain provider network stability and fiscal
11 predictability. One (1) assertive community treatment provider shall be
12 selected per region to provide assertive community treatment services.

13 (3) For services reimbursed under the state medicaid plan, all services
14 furnished as part of assertive community treatment shall be reimbursed only
15 through the assertive community treatment aggregated rate and shall not be
16 separately billed or reimbursed under any other medicaid benefit or service
17 category. Nothing in this subsection shall be construed to prohibit the use
18 of other state or non-medicare funding sources to support assertive commu-
19 nity treatment services for individuals not eligible for medicaid, provided
20 that duplicate payment for the same service does not occur.

21 56-2506. RULEMAKING AUTHORITY. The department of health and welfare
22 shall promulgate rules, subject to legislative approval, pursuant to chap-
23 ter 52, title 67, Idaho Code, as necessary to implement the provisions of
24 this chapter, including provider certification standards, reimbursement
25 rates, reporting requirements, and compliance monitoring, consistent with
26 federal medicaid requirements and the adopted assertive community treatment
27 fidelity model.

28 SECTION 4. An emergency existing therefor, which emergency is hereby
29 declared to exist, this act shall be in full force and effect on and after its
30 passage and approval.